

Assessing Patient's Suitability for Electrical Nerve Stimulation Therapy

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Tracking Information

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Manual Section Title

Assessing Patient's Suitability for Electrical Nerve Stimulation Therapy

Version Number

2

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06/19/2006

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06/19/2006

Description Information

Benefit Category

Incident to a physician's professional Service

Outpatient Hospital Services Incident to a Physician's Service

Outpatient Physical Therapy Services

Physicians' Services

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Indications and Limitations of Coverage

Electrical nerve stimulation is an accepted modality for assessing a patient's suitability for ongoing treatment with a transcutaneous or an implanted nerve stimulator.

Accordingly, program payment may be made for the following techniques when used to determine the potential therapeutic usefulness of an electrical nerve stimulator:

A. Transcutaneous Electrical Nerve Stimulation (TENS)

This technique involves attachment of a transcutaneous nerve stimulator to the surface of the skin over the peripheral nerve to be stimulated. It is used by the patient on a trial basis and its effectiveness in modulating pain is monitored by the physician, or physical therapist. Generally, the physician or physical therapist is able to determine whether the patient is likely to derive a significant therapeutic benefit from continuous use of a transcutaneous stimulator within a trial period of 1 month; in a few cases this determination may take longer to make. Document the medical necessity for such services which are furnished beyond the first month. (See [§160.13](#)  for an explanation of coverage of medically necessary supplies for the effective use of TENS.)

If TENS significantly alleviates pain, it may be considered as primary treatment; if it produces no relief or greater discomfort than the original pain electrical nerve stimulation therapy is ruled out. However, where TENS produces incomplete relief, further evaluation with percutaneous electrical nerve stimulation may be considered to determine whether an implanted peripheral nerve stimulator would provide significant relief from pain.

Usually, the physician or physical therapist providing the services will furnish the equipment necessary for assessment. Where the physician or physical therapist advises the patient to rent the TENS from a supplier during the trial period rather than supplying it himself/herself, program payment may be made for rental of the TENS as well as for the services of the physician or physical therapist who is evaluating its use. However, the combined program payment which is made for the physician's or physical therapist's services and the rental of the stimulator from a supplier should not exceed the amount which would be payable for the total service, including the stimulator, furnished by the physician or physical therapist alone.

B. Percutaneous Electrical Nerve Stimulation (PENS)

This diagnostic procedure which involves stimulation of peripheral nerves by a needle electrode inserted through the skin is performed only in a physician's office, clinic, or hospital outpatient department. Therefore, it is covered only when performed by a physician or incident to physician's service. If pain is effectively controlled by percutaneous stimulation, implantation of electrodes is warranted.

As in the case of TENS (described in subsection A), generally the physician should be able to determine whether the patient is likely to derive a significant therapeutic benefit from continuing use of an implanted nerve stimulator within a trial period of 1 month. In a few cases, this determination may take longer to make. The medical necessity for such diagnostic services which are furnished beyond the first month must be documented.

NOTE: Electrical nerve stimulators do not prevent pain but only alleviate pain as it occurs. A patient can be taught how to employ the stimulator, and once this is done, can use it safely and effectively without direct physician supervision. Consequently, it is inappropriate for a patient to visit his/her physician, physical therapist, or an outpatient clinic on a continuing basis for treatment of pain with electrical nerve stimulation. Once it is determined that electrical nerve stimulation should be continued as therapy and the patient has been trained to use the stimulator, it is expected that a stimulator will be implanted or the patient will employ the TENS on a continual basis in his/her home. Electrical nerve stimulation treatments furnished by a physician in his/her office, by a physical therapist or outpatient

clinic are excluded from coverage by §1862(a)(1) of the Act. (See §[160.7](#) for an explanation of coverage of the therapeutic use of implanted peripheral nerve stimulators under the prosthetic devices benefit.) See §[160.27](#) for an explanation of coverage of the therapeutic use of TENS under the durable medical equipment benefit.

Transmittal Information

Transmittal Number

48

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/Downloads/R48NCD.pdf>

Revision History

03/2006 - Removal of cross reference. Effective date: 06/19/2006. ([TN 48](#)) (CR4278)

07/1995 - Determined that TENS covered under durable medical equipment benefit rather than prosthetic device benefit. Effective date 08/07/1995. (TN 78)

09/1988 - Added cross-reference omitted from previous transmittal. Effective date NA. (TN 33)

06/1988 - Added cross-references to section 45-25. Effective date 07/14/1988. (TN 26)

Additional Information

Other Versions

Title	Version	Effective Between	
Assessing Patient's Suitability for Electrical Nerve Stimulation Therapy	2	06/19/2006 - N/A	You are here
Assessing Patient's Suitability for Electrical Nerve Stimulation Therapy	1	08/07/1995 - 06/19/2006	View